

Office Visit 8/29/2024
PAV WH Multidisciplinary
Oncology Clinic

Provider: Torres Yee, Jennifer C, MD (Hematology and Oncology)
Primary diagnosis: Non-small cell cancer of left lung (CMS/HCC)
Reason for Visit: Follow-up

Progress Notes

Torres Yee, Jennifer C, MD (Fellow) • Oncology



DIVISION OF HEMATOLOGY & ONCOLOGY

CLINIC NOTE

DOS:8/29/24

CHIEF COMPLAINT: follow up

HISTORY OF PRESENTING ILLNESS: [REDACTED] 79 y.o. male presents follow up and discuss treatment options after he established Oncology care at our Clinic 8/1/24 for his new diagnosis of RUL SCCa s/p bronch 7/17/24.

CURRENT TREATMENT:

- Consideration for possible CT/IO [no consent] + cont only pXRT to chest/LN +/- brain RT pending repeat MRI brain inpatient

TODAY, follow up evaluation 8/29/24. He is accompanied by family during visit.

Pt reported feeling "ok" today but he is disoriented mostly to time during visit.

GOC discussed during visit and focus is QoL after MRI results.

They remain interest to pursue tx options in deemed candidate.

New progressive worsening of right sided weakness upper/lower extremities after recurrent falls with disorientation, and numbness [chronic] that is also worsened. Son reports declined PS during this time. outside evaluation in ED for stroke reportedly ok but no recs found.

Noted BP 180/95 during visit with repeat 160/78 after/ rate controlled Afib [chronic].

ROS negative.

HEMATOLOGY / ONCOLOGY HISTORY:

Oncology History Overview Note

Patient is from CYNTHIANA KY 41031

PMHx:

- Afib on Eliquis (off ASA), BB (off amiodarone), severe chronic CAD s/p PCI (last early 2024), HLD, HTN, DM 2, Heavy tobacco smoking (4PPD x 40 yrs); quit smoking > 6 wks ago. NO home O2.

Kidney mass (UK: Dr Hensley).

- right 3.3 cm renal mass s/p biopsy and cryoablation with IR on 5/15/23. Pathology remarkable for oncocytic cells. CT renal mass and CXR 3/7/24 was NER or RD. Deferred to his PCP and a vascular surgeon for his worsening RLE weakness and cold extremity. **. RTC in 1 year with CT RMP, CXR**

Lung mass:(UK Dr Maskey, Dr Yang).

- Patient has been noted to have a RUL opacity since 3/2023. It was not FDG avid on a PET 3/2023. He underwent EBUS FNA of level 7 as well as RUL BAL and RUL transbronchial biopsy 4/17/23=benign and did not demonstrate any organisms on culture. He had ongoing surveillance of his lung nodules and RUL mass 7/2023, 10/2023, and 6/2024, and due to changes in size and appearance of this right upper lobe mass he was referred to IP (Dr Maskey). Given his history, mass appearance, and recent changes, concerns for a malignant process and possibly a metastatic process related to his prior renal mass by Dr Maskey recommended further evaluation by medical oncology, Dr Patrick Hensley, MTB discussion.

- 7/17/2024. Underwent evaluation of RUL mass w/ bronchoscopy with biopsy = "RUL bx showed it is not specific for a primary site. Considerations include metastasis from breast or urothelial tract as well as primary or metastatic non-small cell carcinoma (with squamous differentiation). Clinical correlation is required.

8/1/24: GMOC(Dr Hao/Dr TorresYee). initial evaluation. clinically stable from chronic sx (dyspnea). Asx (renal mass).

8/8/24 FDG-PETCT (staging): GS. No hydronephrosis Diverticulosis. No pathologically enlarged or pet avid LN. Bladder wall thickening. Prostate calcifications (likely age related-- **PSA ?**). No suspicious pet avid masses in abd/pel/hepato-pancreatobiliary. No suspicious pet avid bony lesions. CHRONIC bilateral rib fractures (ASX). C3-4 hardware.

Multiple pet avid bilateral lung nodule. Sublet pet avid right hilar region over R-PA (SUV 4). No mediastinal, hilar, or axillary LAD.

R-kidney changes c/w cryoablation.

- RUL mass 4.8 x 2.2 cm [SUV 9.5]- was prior 4.1 x 2.1 cm on prior CT, (series 4, image 118).
- RUL nodule 1.9 cm, SUV 2. - stable from prior CT, (series 4, image 120).
- LUL nodule 2 cm, SUV 2.9, previously 1.8 cm, (series 4, image 123).

- **IMPRESSION:**

- Multiple bilateral FDG avid pulmonary nodules and/or masses with overall slight increase in size from comparison CT, suspicious for malignancy. Dominant hypermetabolic mass in the right upper lobe.
- Questionable FDG avid right hilar lymph node metastasis. Otherwise, no pathologically enlarged or hypermetabolic mediastinal or hilar lymph nodes.
- No evidence of FDG avid distant metastatic disease.
- Previously ablated right renal mass better evaluated on renal mass CT protocol 3/6/2024.

Planned to discuss in Lung MTB, but Case discussed early by RadOnc/Pulm 8/8/24 while awaiting further to complete staging eval; per notes, discussion determined pt to have likely "potential metastatic renal mass to lung ": Dr. Bernard / Dr Yang - SBRT to RUL mass followed by systemic therapy evaluation by MedOnc discussed. The following RT plan considered:

- RUL masslike opacity[[3.2 x 5.1 cm,] = SBRT. [60Gy/15 to the right side lesion based on safety considerations-- Clinical toxicity and fatal lung hemorrhage most observed when tumors were located ≤ 1 cm from the trachea or mainstem bronchi (Group A: 46% of all cases). (60Gy/8 (7.5 Gy/Fx BED = 105)
- New 1.5 cm right hilar LN (too close to bronchus) - not SBRT candidate
- A nodule on the left side of lung ?RT plan - > if no RT planned, will plan to tx systemically if pursued options

***CARIS sent 7/17/14: HER2 negative, TP53 (exon 7) ALK- MSI-stable . Exon 18 and Exon 21 mutation [G719A and G824A] in EGFR**

POC: RTC to discuss treatment options after patient completes: ordered 8/1/24 baseline ECHO- ordered 8/1/24 baseline labs (CMP, CBCD, PSA).

PAST HISTORY: MEDICAL / SURGICAL / FAMILY / SOCIAL / ALLERGIES

Past Medical History:

Diagnosis	Date
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- Allergies
- Atrial fibrillation (CMS/HCC)
- Coronary artery disease
- Diabetes (CMS/HCC)
- Diabetes (CMS/HCC)
- Heart problem
- High cholesterol
- Hypertension
- Lung trouble
- Shortness of breath

Past Surgical History:

Procedure	Laterality	Date
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- CARDIAC STENT
- COLONOSCOPY
- KIDNEY SURGERY
- LUNG BIOPSY
- OTHER SURGICAL HISTORY
leg surgery.

Family History

Problem	Relation	Name	Age of Onset
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- Diabetes Mother
- Cancer Mother
- Diabetes Father
- Cancer Father
- Breast cancer Sister
- Anesthesia problems Neg Hx
- Malignant Hyperthermia Neg Hx

Social History

Socioeconomic History

- Marital status: Married
Spouse name: Not on file
- Number of children: Not on file
- Years of education: Not on file
- Highest education level: Not on file

Occupational History

- Not on file

Tobacco Use

- Smoking status: Former
Types: Cigarettes
- Smokeless tobacco: Never
- Tobacco comments:
Quit 25 yrs ago.

Vaping Use

- Vaping status: Never Used

Substance and Sexual Activity

- Alcohol use: Never
- Drug use: Never
- Sexual activity: Not on file

Other Topics Concern

- Not on file

Social History Narrative

- Not on file

Social Determinants of Health

Financial Resource Strain: Not on file

Food Insecurity: No Food Insecurity (1/19/2024)

Received from Catholic Health Initiatives

Food Insecurity

- : Not on file
- : Not on file

Transportation Needs: Not on file

Physical Activity: Not on file

Stress: Not on file

Social Connections: Low Risk (1/19/2024)

Received from Catholic Health Initiatives

Family and Community Support

- : Not on file
- : Not on file

Intimate Partner Violence: Unknown (10/10/2023)

Received from Baptist Healthcare System

Abuse Screen

- Unsafe at Home or Work/School: Not on file
- Feels Threatened by Someone?: Not on file
- Does Anyone Keep You from Contacting Others or Doing Things Outside the Home?: Not on file
- Physical Sign of Abuse Present: Not on file

Housing Stability: Low Risk (1/19/2024)

Received from Catholic Health Initiatives

Housing Stability

- : Not on file
- : Not on file

Allergies**Allergen****Reactions**

- Penicillins
 - Hydrocodone-Acetaminophen
- Anaphylaxis
Other - please document in the comment field

*Heart attack and swelling**Patient is not allergic to tylenol***VITAL SIGNS:**

	7/17/2024 2:00 PM	7/17/2024 2:15 PM	8/1/2024 9:24 AM	8/1/2024 9:57 AM	8/9/2024 2:59 PM	8/9/2024 3:07 PM	8/29/2024 8:35 AM
Vitals							
Systolic	152 †	158 †	165 †	165 †	173 †	173 †	180 †
Diastolic	75 †	64 †	74 †	68 †	66 †	72 †	70 †
Heart Rate	75	77	52		60		67
Temp		36.4 C	36.4 C		36.3 C		36.3 C
Resp	21	19			16		
Height (cm)			179.1 cm		177.8 cm		177.8 cm
Weight (kg)			95.1 kg		95.3 kg		96.4 kg
BMI			29.66 kg/m ²		30.15 kg/m ²		30.49 kg/m ²
BSA (m ²)			2.17 m ²		2.17 m ²		2.18 m ²
Visit Report			Report	Report			Report

Body surface area is 2.18 meters squared. Body mass index is 30.49 kg/m². @WEIGHTCHANGE@

1: Restricted in physically strenuous activity but ambulatory and able to do light work and 2: Ambulatory and capable of all self-care but unable to work. Up & about >50% waking hours

PHYSICAL EXAMINATION:

General: Elderly patient in NAD; **fatigued appearing**

HEENT: No pallor, anicteric, no thrush

Heart: S1S2, **Afib**, no audible murmur. **Edema ([ddi not take home diuretic this AM])**

Lungs: Clear to auscultation bilaterally. Normal WOB.

Abdomen: soft, non tender, ND; organomegaly not palpable
Lymphatics: No palpable cervical, axillary or inguinal adenopathy
Skin/Extremities: No edema, No rash. +**Bruising**

Neuro: AAOX2 (disoriented to time) - gossly w+ FND; RUE/ELL weakness and sensory deficits.

Psych: in good spirits, cooperative and interactive; hopeful

RESULTS:

All results were personally reviewed and interpreted by me. Discussed results with the patient and the attending supervisor attesting this note.

No labs prior visit

Final Diagnosis (no units)

Date/Time	Value
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07/17/2024 1209

A. BRONCHIAL BRUSHING, RIGHT UPPER LOBE:
- POSITIVE FOR MALIGNANCY.

B. RIGHT UPPER LOBE, BRONCHOALVEOLAR LAVAGE:
- POSITIVE FOR MALIGNANCY.
- GMS STAIN NEGATIVE FOR ORGANISMS.

07/17/2024 1209

A. LUNG, RIGHT UPPER LOBE, ION ROBOTIC NAVIGATIONAL FINE NEEDLE
ASPIRATION:
- POSITIVE FOR MALIGNANCY, NON-SMALL CELL CARCINOMA. SEE
COMMENT.

B. LYMPH NODE, STATION 7, ENDOBRONCHIAL ULTRASOUND-GUIDED FINE
NEEDLE ASPIRATION:
- SCANT HEMODILUTE LYMPHOID TISSUE.

C. LUNG, RIGHT UPPER LOBE, TRANSBRONCHIAL BIOPSY:
- NON-SMALL CELL CARCINOMA. SEE COMMENT.

Comment (no units)

Date/Time	Value
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07/17/2024 1209

See also C24-9362 for more information.

07/17/2024 1209

The immunistochemical staining pattern of this tumor, summarized below, is not specific for a primary site. Considerations include metastasis from breast or urothelial tract as well as primary or metastatic non-small cell carcinoma (with squamous differentiation). Clinical correlation is required. In an effort to conserve tissue, no additional stains (including PD-L1) will be performed at this time unless requested.

Addendum (no units)

Date/Time	Value
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05/15/2023 1324

This addendum is to document the provided clinical history There is no change in the final diagnosis.

Right renal mass

IF THIS BIOPSY IS REPRESENTATIVE OF THE ENTIRE LESION, IT COULD BE CONSISTENT WITH AN ONCOCYTOMA. HOWEVER, SEVERAL RENAL CELL CARCINOMA SUBTYPES MAY SHOW FOCAL AREAS WITH ONCOCYTIC FEATURES OF THE KIND PRESENT IN THIS CASE. CLINICAL CORRELATION IS ADVISED.

ASSESSMENT/PLAN: [REDACTED] is a 79 y.o. male presents to establish Oncology care in our Clinic for his new diagnosis of RUL SCCa s/p bronch 7/17/24

Suspect new biopsy proven SCCa of RUL2/2 Enlarging LUL lung mass (TTF-, p40+, TPS 90%) in the setting of a R-kidney biopsy proven oncocytoma (less likelihood spread to the lungs; still possible but extremely rare)

-3/2023: outside PETCT: non-avid lung mass (non-avid kidney mass

-7/31/24: MRI brain: questionable dural enhancement, no brain/skull lesions (asx).

-Baseline ECOG 1/2 since COVID19 infection in 2023, uses rolling walker since then.

-7/17/24RUL bx showed it is not specific for a primary site.

- "Considerations include metastasis from breast or urothelial tract as well as primary or metastatic non-small cell carcinoma (with squamous differentiation). Clinical correlation is required." -Renal oncocytomas usually have an excellent prognosis and are not associated with an aggressive clinical course; however, there can occasionally be metastasis to liver and bone and fatalities have occurred but these tumors very rarely go to the lung. Surgical excision is curative if there have not been metastases (if surgical candidate). On further review of the literature, Despite the classic appearance, there are several entities that mimic oncocytoma so diagnosis remains a challenge. Diagnosis is primarily made based on histologic appearance and, in challenging cases, with immunohistochemistry. Rarely cytogenetic studies are useful. Rearrangements of *CCND1* gene are common which were not reported in NGS sent 7/17/24. Based on final review of his final IHC on RUL biopsy results, and behavior of the above we would favor more a lung primary malignancy- likely SCCa (+p40

partial). Recommend further evaluation with FDG PETCT scan, ECHO as initially planned and further discussion in lung MTB once "ALL" studies are complete.

Based on NSG results from 7/17/24 from RUL biopsy, clinical benefits with the following has been reported in patients with TPS 90 (cemiplimab, pembro- level1), PDL1+2 (nivo/ipili combo- level1), PDL1 90% (adjuvant atezo, cemiplimab- level 1), EGFR exon 18 (p.g719a) (afatinib, osimertinib-level 2; erlotinib, dacomitinib, gefitinib- level 3), TMB high (pembro- level 2).

- **ECHO- still needs**
- **-PETCT8/8/24**
- **-CMP, CBCD, HIV, HCV, TSH, B12, folate**

PLAN: 8/29/24:

- **Admit to HM 11/12 teams. Medical oncology consult inpatient. He was recommended admission to UK for urgent neurology evaluation, rule out brain mets. Consideration for possible CT/IO [no consent] + cont only pXRT to chest/LN +/- brain RT pending repeat MRI brain inpatient. his MRI brain 7/31 showed equivocal non-specific changes; now clinically p/w suspicion for possible progression**
 - **Completing pXRT to chest/LN x2 fxs- planned for tx after visit s outp - (planned for total x15 fxs)- not contraindicated with concurrent CT based on mutations**
 - **PT evaluation**
- RTC 1-2 weeks after DC from hospital**

Jennifer Torres Yee, MD, MSc

Hematology and Oncology Fellow PGY-6

CHANDLER MARKEY

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Electronically Signed by: Jennifer C Torres Yee, MD - 8/29/2024 - 9:23 AM

I, Zhonglin Hao, MD, saw and evaluated the patient. I discussed the case with the medical student and resident/fellow and agree with the findings and plan as documented. I personally performed the Exam and Medical Decision Making.

Instructions

After Visit Summary (Automatic SnapShot taken 9/3/2024)

Additional Documentation

Vitals: BP **167/78** ! (Abnormal) (BP Location: Left arm, Patient Position: Sitting) Pulse 67
 Temp 36.3 °C (97.4 °F) Ht 1.778 m (5' 10") Wt 96.4 kg (212 lb 8.4 oz) SpO2 96% BMI 30.49 kg/m²
 BSA 2.18 m² Pain Sc 2 (Loc: Knee)

Flowsheets: NCCN Distress Thermometer, Vitals

SmartForms: AMB SF STEADI FALL RISK

Communications

 Letter sent to Jennie L Gardner, APRN Provider Notes
Sent 9/3/2024

No questionnaires available.

Orders Placed

CBC and differential (Resulted 8/29/2024, Abnormal)

Comprehensive metabolic panel (Resulted 8/29/2024, Abnormal)

Medication Changes

As of 8/29/2024 8:39 AM

None

Medication List at End of Visit

As of 8/29/2024 8:39 AM

	Refills	Start Date	End Date
albuterol (2.5 MG/3ML) 0.083% nebulizer solution Take 3 mL (2.5 mg) by nebulization 4 (four) times a day if needed for wheezing or shortness of breath. - Nebulization Patient-reported medication	—	12/30/2022	8/29/2024
amiodarone (Pacerone) 200 MG tablet Take by mouth 2 (two) times a day. - Oral Patient not taking: Reported on 6/24/2024 Patient-reported medication	—		8/29/2024
Eliquis 5 MG tablet Take 1 tablet (5 mg) by mouth 2 (two) times a day. - Oral Patient-reported medication	—	2/18/2023	—
aspirin 81 MG EC tablet Take 1 tablet (81 mg) by mouth 1 (one) time each day in the morning. - Oral Patient-reported medication	—		—
bumetanide (Bumex) 1 MG tablet Take 1 tablet (1 mg) by mouth 1 (one) time each day in the morning. - Oral Patient-reported medication	—	5/30/2023	—
clopidogrel (Plavix) 75 MG tablet Take 75 mg by mouth 1 (one) time each day. - Oral Patient not taking: Reported on 6/24/2024 Patient-reported medication	—	2/22/2023	8/29/2024
CYANOCOBALAMIN PO Take 1 tablet by mouth 1 (one) time each day. - Oral Patient-reported medication	—		—
cyclobenzaprine (Flexeril) 5 MG tablet Take 1 tablet (5 mg) by mouth 1 (one) time each day. - Oral Patient not taking: Reported on 6/24/2024	—	9/6/2023	8/29/2024

	Refills	Start Date	End Date
Patient-reported medication			
Fluticasone-Umeclidin-Vilant (TRELEGY ELLIPTA IN) Inhale 1 Inhalation if needed. - Inhalation	—		8/29/2024
Patient-reported medication			
furosemide (Lasix) 20 MG tablet Take 20 mg by mouth 1 (one) time each day. - Oral Patient not taking: Reported on 6/24/2024	—	2/18/2023	8/29/2024
Patient-reported medication			
gabapentin (Neurontin) 300 MG capsule Take 2 capsules (600 mg) by mouth 1 (one) time each day. - Oral Patient not taking: Reported on 6/24/2024	—	6/19/2023	8/29/2024
Patient-reported medication			
glimepiride (Amaryl) 1 MG tablet Take 1 tablet (1 mg) by mouth 1 (one) time each day before breakfast. - Oral	—	2/12/2023	—
Patient-reported medication			
ipratropium-albuterol (Duo-Neb) 0.5-2.5 mg/3 mL nebulizer solution Take 3 mL by nebulization 2 (two) times a day. - Nebulization	—	6/6/2024	—
Patient-reported medication			
losartan (Cozaar) 100 MG tablet Take 1 tablet (100 mg) by mouth 1 (one) time each day in the morning. - Oral	—	2/1/2023	—
Patient-reported medication			
metFORMIN (Glucophage) 500 MG tablet Take 1 tablet (500 mg) by mouth 2 (two) times a day with meals. - Oral	—	10/6/2022	8/31/2024
Patient-reported medication			
Metoprolol Succinate			
metoprolol succinate XL (Toprol-XL) 50 MG 24 hr tablet Take 50 mg by mouth 1 (one) time each day. - Oral Patient not taking: Reported on 6/24/2024	—	2/22/2023	8/29/2024
Patient-reported medication			
metoprolol succinate XL (Toprol-XL) 25 MG 24 hr tablet Take 1 tablet (25 mg) by mouth 1 (one) time each day in the morning. - Oral	—	4/10/2024	9/1/2024
Patient-reported medication			
metoprolol tartrate (Lopressor) 50 MG tablet Take 1 tablet (50 mg) by mouth. - Oral	—	1/19/2023	8/29/2024
Patient-reported medication			
Multiple Vitamins-Minerals (multivitamin with minerals) tablet Take 1 tablet by mouth 1 (one) time each day. - Oral	—		—
Patient-reported medication			
nutritional drink (Boost) liquid liquid Take 237 mL by mouth 2 (two) times a day. - Oral Notes to Pharmacy: Please keep on file; pt will call to fill	5	8/1/2024	8/29/2024
Patient-reported medication			
simvastatin (Zocor) 40 MG tablet Take 1 tablet (40 mg) by mouth every night. - Oral	—	2/12/2023	9/1/2024
Patient-reported medication			

Visit Diagnoses

Primary: **Non-small cell cancer of left lung (CMS/HCC)** C34.92
 Altered mental status, unspecified altered mental status type R41.82

